Medical Report — Chen Yuanhuan

Orbital inflammation work-up · Shenzhen & Hong Kong · Mar 2026

Patient
Chen Yuanhuan · Male · 72 yrs

Phone
18676473617

Chief complaint
Left eyelid swelling >3 months

Working diagnosis
Orbital inflammation (OS)

Current Medications (Supplement)

> Patient is currently taking **two of three** chronic “three highs” medications (hypertension / hyperlipidemia regimen): **Amlodipine 10 mg** and **Atorvastatin (Lipitor) 10 mg**, both once daily — prescribed in Hong Kong (Dec 2025).

Timeline

Date	Event
~Dec 2025	Left eyelid swelling began; TCM treatment without improvement
Before Mar 17	Lacrimal duct irrigation — no obvious abnormality
Mar 17, 10:14	**Shenzhen Eye Hospital** — ophthalmology outpatient visit
Mar 17	**Overseas Chinese Town Hospital** — orbit CT (plain scan)
Mar 17, 12:59	**Futian Rheumatology Hospital (Xiangmihu)** — rheumatology/immunology labs ordered
Mar 17, 13:12	Blood & serum samples collected
Mar 17, 14:31	CBC + ESR results released
Mar 17, 15:36	CRP, complement, immunoglobulins released
Mar 18	ANA, autoantibody panel (22 items), antiphospholipid panel, RA early panel released
Mar 18, 16:30	IgG4 sample received — Guangzhou KingMed
Mar 19, 17:19	IgG4 result released

Clinical Encounters

Mar 17 — Shenzhen Eye Hospital (Outpatient)

- **History:** 3-month left eyelid swelling; bloody and purulent discharge on compression; no eye pain. Past: bilateral cataract surgery, bladder cancer surgery 6 years ago, hypertension.
- **Exam:** Left upper/lower eyelid swelling (+); no palpable mass at lid margin; clear cornea; normal eye movements. Visual acuity OD 0.6 / OS 0.8.
- **Plan:** Orbit CT (Overseas Chinese Town Hospital); rheumatology/immunology work-up + IgG4 (Xiangmihu); prednisone 25 mg QD, potassium chloride, omeprazole; follow up as needed.

Mar 17 — Overseas Chinese Town Hospital (CT Orbit)

- Left periorbital soft-tissue swelling and thickening
- Left levator palpebrae superioris thickened (~1.6 mm)
- Bilateral lens thinning (post cataract surgery)
- Optic nerves, extraocular muscles, orbital bones — no significant abnormality
- ~2 mm subcutaneous calcification, right forehead
- Mucosal thickening: bilateral frontal & ethmoid sinuses, right maxillary sinus

Impression: Left periorbital soft-tissue swelling; left levator muscle thickening; post-operative lenses; right frontal subcutaneous calcification; sinusitis (bilateral frontal/ethmoid, right maxillary).

Laboratory Results Summary

All samples collected Mar 17, 13:12 · Clinical diagnosis: orbital disease · Requesting physician: Huang Xinmin

Inflammation
CRP 4.00 mg/L · ESR 8 mm/h — Normal

Immunoglobulins
IgG 11.92 · IgA 2.71 · IgM 1.31 · IgG4 1.91 g/L — All normal

Complement
C3 0.99 · C4 0.29 g/L — Normal

ANA
— Negative

Rheumatoid panel
RF, anti-CCP, AKA, GPI — Negative / normal

Vasculitis panel
p-ANCA, c-ANCA, MPO, PR3, GBM — Negative

Connective tissue antibodies
20-item panel (incl. dsDNA, Sm, SSA/SSB, Scl-70, Jo-1, etc.) — All negative

Antiphospholipid syndrome (6 items)
— Within range

CBC — only abnormality

Test	Result	Reference	Status
Large platelet count	27	30–90 $\times 10^9$/L	Low
All other CBC parameters (WBC, Hb, PLT, differentials) — normal			

Clinical Synthesis

- **Confirmed findings:** Persistent left eyelid/periorbital swelling (3+ months) with purulent discharge; CT confirms left periorbital soft-tissue swelling and levator muscle thickening.
- **Not supported by labs:** IgG4-related disease, typical rheumatoid arthritis, common connective tissue diseases, ANCA-associated vasculitis, antiphospholipid syndrome; no systemic inflammatory marker elevation.
- **Incidental:** Paranasal sinusitis; small right frontal subcutaneous calcification; isolated low large-platelet count (likely minor — repeat if needed).
- **Treatment started (Mar 17):** Prednisone 25 mg/day with potassium and omeprazole prophylaxis.

> **Drug interaction note:** New prednisone may affect blood pressure and glucose/lipids. Inform treating physicians of both Shenzhen prescriptions and Hong Kong chronic medications (Amlodipine, Atorvastatin) at follow-up.

Follow-up Points

- Monitor response of eyelid swelling to prednisone
- Discuss whether MRI, biopsy, or culture is needed given purulent discharge despite no palpable lid mass
- Bring this report and all original lab/imaging documents to next visit

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For reference only — final diagnosis and treatment must be confirmed by the treating physician.